

PERSISTENCE, DISCONTINUATION AND SPECIALTY PHARMACIST INTERVENTIONS IN PATIENTS RECEIVING DROXIDOPA FOR THE MANAGEMENT OF ORTHOSTATIC HYPOTENSION

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BACKGROUND

- Droxidopa improves blood pressure and reduces the number of falls in patients with neurogenic orthostatic hypotension (OH).¹
- Reported 6-month discontinuation rates are between 6 and 25%, most commonly due to adverse events or reduction in benefit.^{2,3,4}
- Integrated neurology specialty pharmacists play a crucial role in management of droxidopa therapy by providing education and clinical interventions.

OBJECTIVES

- PRIMARY** Evaluate persistence, discontinuation and reasons for discontinuation for patients receiving droxidopa
- SECONDARY** Describe number and types of pharmacist interventions

METHODS

- DESIGN** Single-center, retrospective cohort analysis
- INCLUSION** Adult patients prescribed droxidopa by center's outpatient neurology and cardiology clinics with ≥ 3 fills through center's specialty pharmacy from May 2017- April 2020
- EXCLUSION** Patients who did not start droxidopa therapy or passed away within 90 days of initiation
- PERSISTENCE** Measured as time to the first non-persistent event, defined as a coverage gap exceeding 60 days

RESULTS

TABLE 1. PATIENT DEMOGRAPHICS (N=89)

	n (%)
Age, years, mean ± SD	71 ± 8
Gender, male	57 (64)
Race	
Caucasian	76 (85)
African American	11 (12)
Not reported	2 (2)
Insurance type	
Medicare	67 (75)
Commercial	15 (17)
Tricare	5 (6)
Medicaid	2 (2)
Type of orthostatic hypotension	
Parkinson's Disease	36 (40)
Pure autonomic atrophy	29 (33)
Multiple system atrophy	10 (11)
Other*	6 (7)
Non-diabetic autonomic neuropathy	4 (4)
Non-neurogenic orthostatic hypotension	4 (4)

Treatment naïve 58 (65)

*Other diagnoses: amyloidosis, autonomic failure secondary to diabetes, postural orthostatic tachycardia syndrome, diabetic peripheral neuropathy, dopamine beta-hydroxylase deficiency

RESULTS

FIGURE 1. PROBABILITY OF PERSISTENCE ON DROXIDOPA

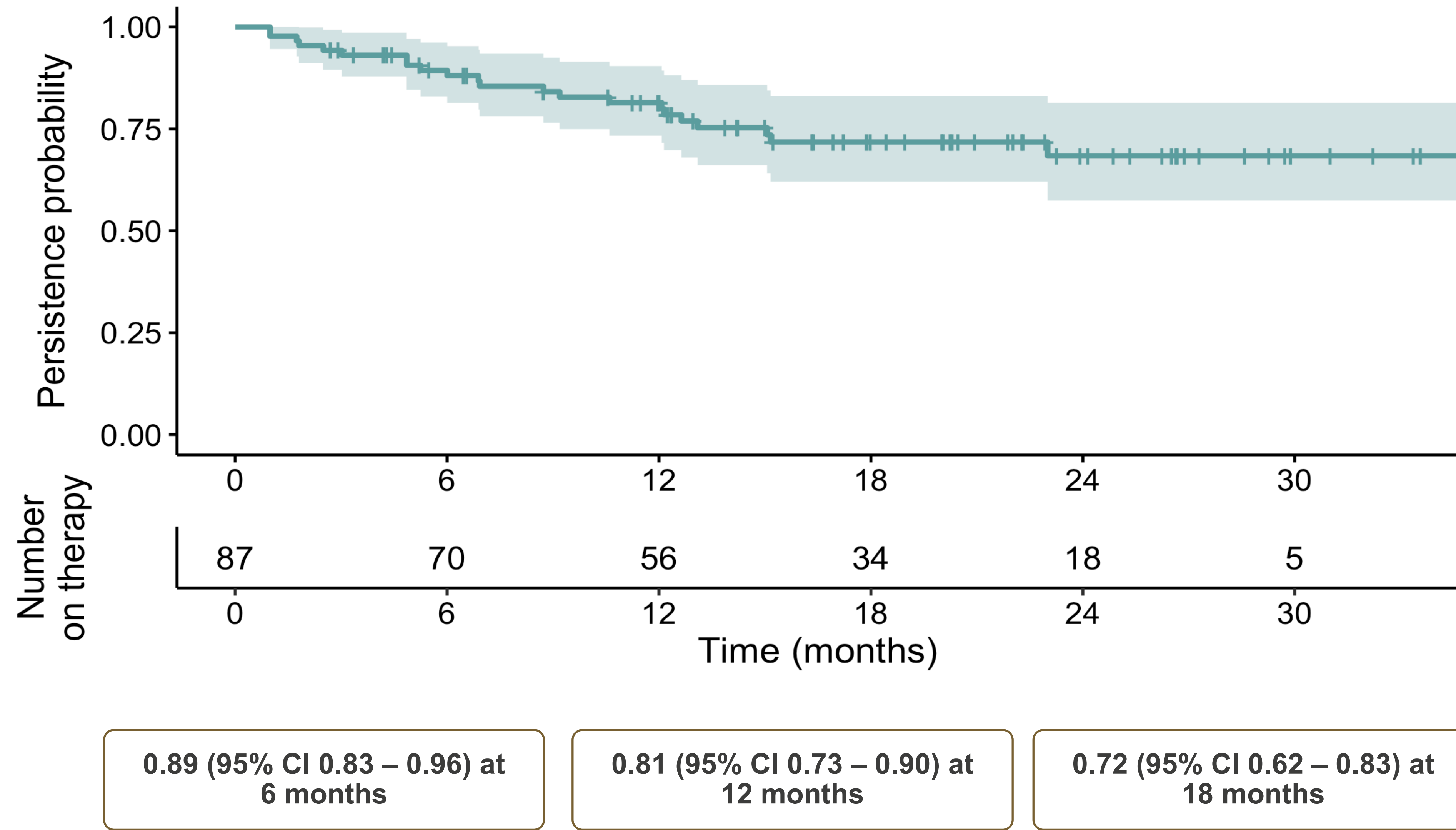
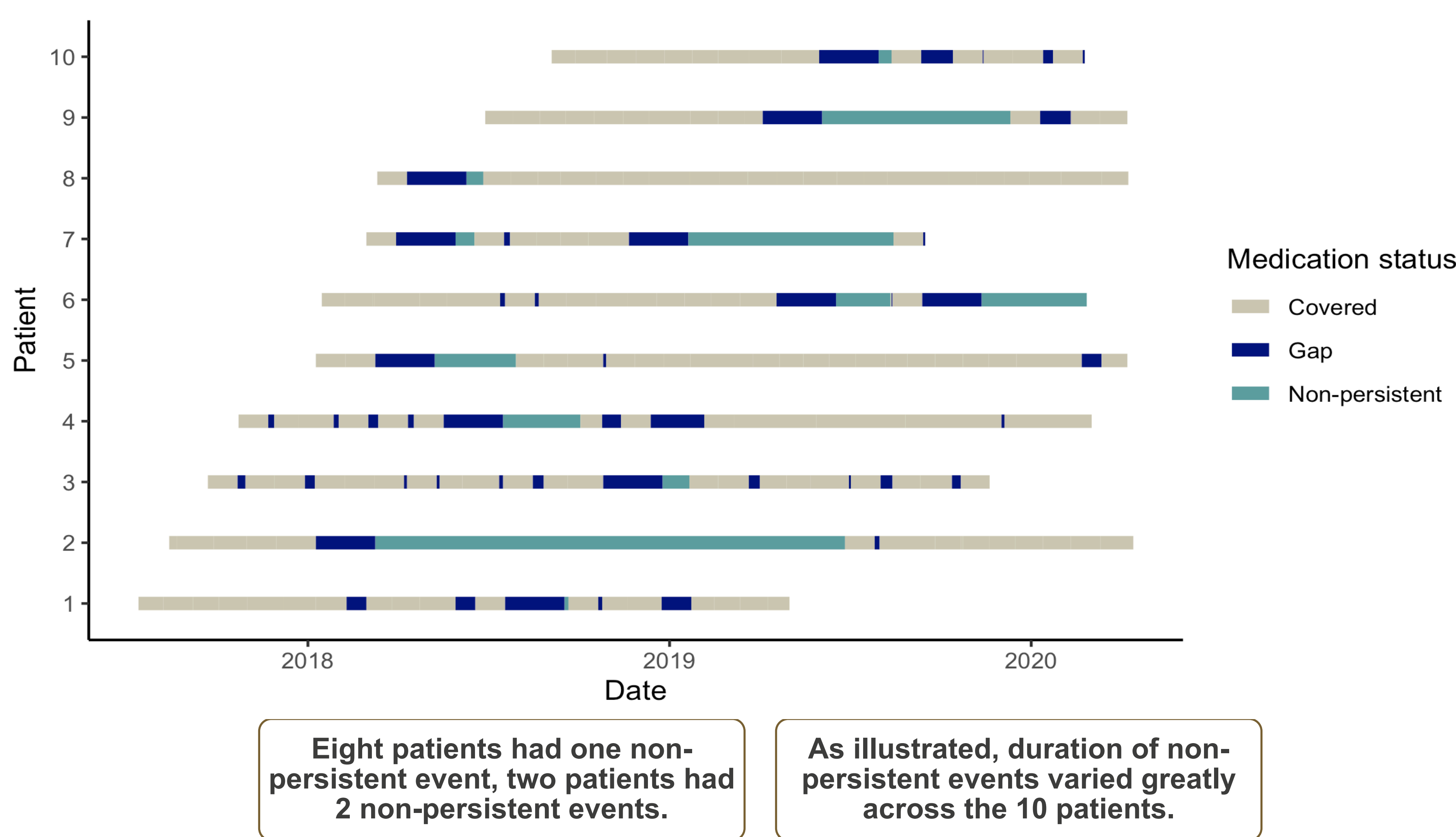


FIGURE 3. LAPSE FOR PATIENTS WITH NON-PERSISTENCE AS FIRST EVENT (N=10)



CONCLUSIONS

- This study found a high probability of persistence within an integrated care model. The discontinuation rate was similar to what is reported in literature^{2,3,4}.
- Specialty pharmacists perform an important role in reviewing medication dosing and drug interactions. Our findings support the integrated specialty pharmacist role in ongoing treatment monitoring.

FIGURE 2. TIME TO FIRST EVENT FOR ALL PATIENTS

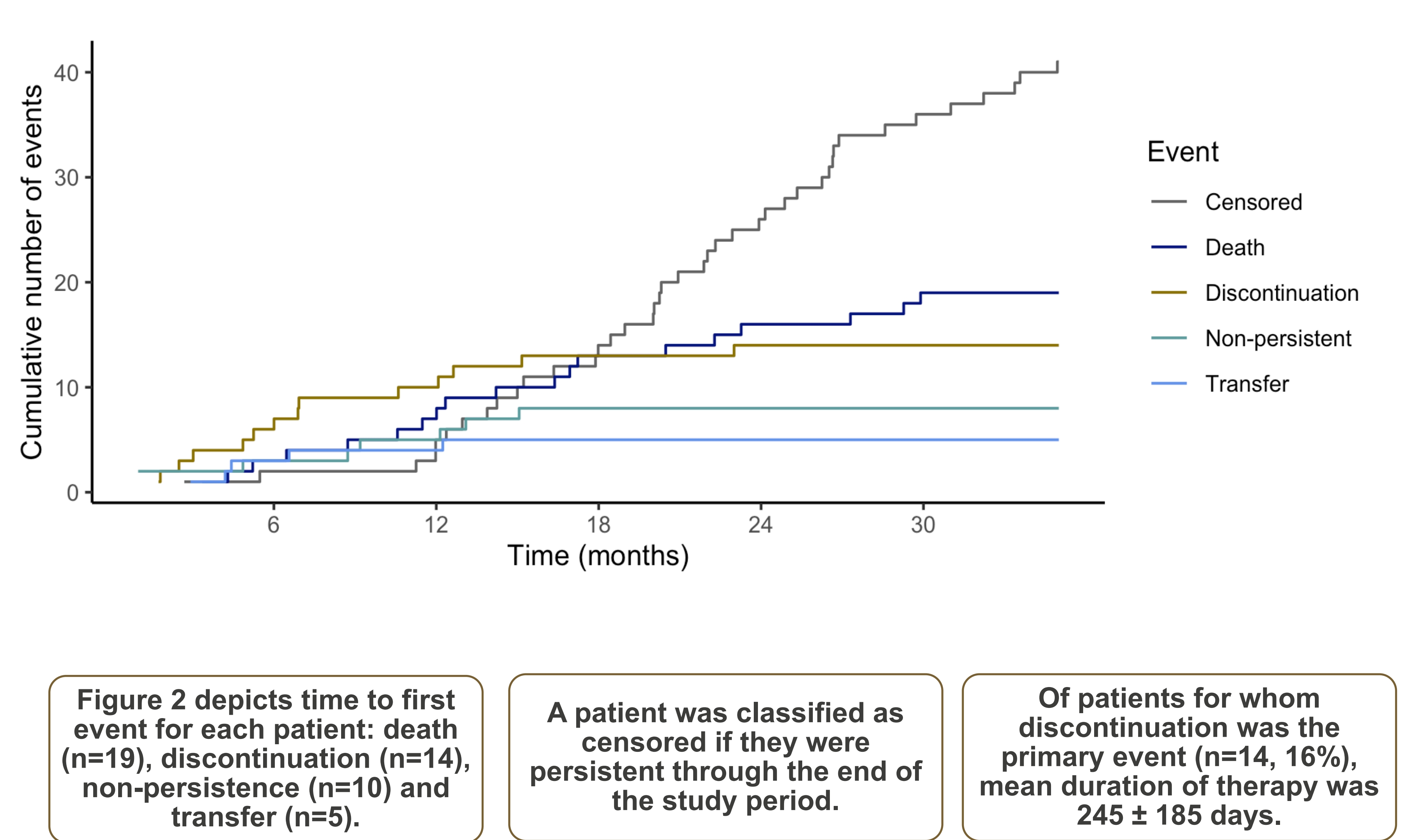
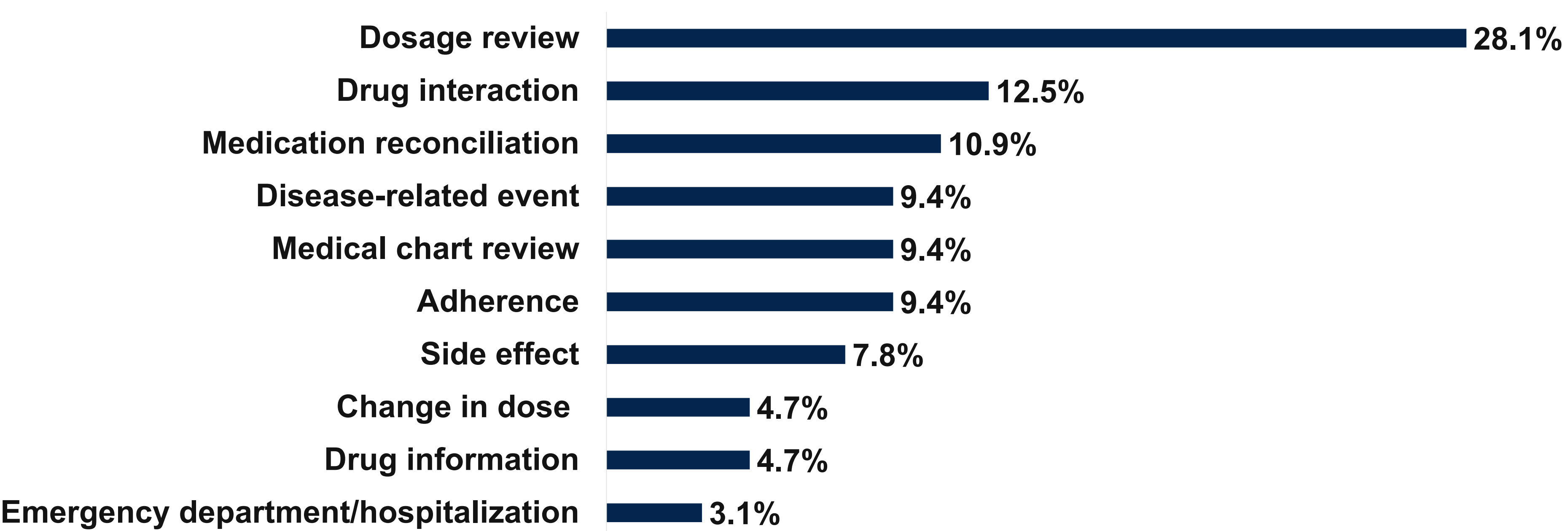


TABLE 2. REASONS FOR ANY DISCONTINUATION (N=24)

	n (%)
Clinical decline	6 (25)
Adverse event	4 (17)
Patient-perceived lack of effectiveness	4 (17)
Financial limitations/cost	4 (17)
Medication no longer appropriate	3 (13)
Non-responsive to refill calls	2 (8)
Administration difficulty	1 (4)

FIGURE 4. PHARMACIST INTERVENTIONS (N=64)



Neurology specialty pharmacists performed 64 interventions via telephone for 34 patients.

As droxidopa therapy was frequently titrated, most interventions were dosage review.